

ST. LUKE'S UNIVERSITY HOSPITAL

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PATIENT MEDICAL REPORT

Report Date: 2026-04-20 | Patient ID: SLH-2026-11054

PATIENT INFORMATION

PATIENT NAME	David Chen	AGE / GENDER	41 Years / Male
PATIENT ID	SLH-2026-11054	ATTENDING PHYSICIAN	Dr. Priya Nair, MD
ALLERGIES	Sulfonamides (urticaria)	SPECIALTY	Pulmonology

CHIEF COMPLAINTS

- Chronic productive cough — 8 weeks; yellow-green sputum
- Dyspnea on moderate exertion; worse over past 10 days
- Wheezing audible to patient, especially at night
- Fever 38.4 °C for 3 days; mild night sweats

VITAL SIGNS

Parameter	Value	Unit	Status
Blood Pressure	118/76 mmHg	mmHg	Normal
Pulse	102 bpm	bpm	HIGH
Temperature	101.1 °F	°F	HIGH
Respiratory Rate	24 /min	/min	HIGH
SpO2	93%	room air	LOW
Peak Flow	55% predicted	%	LOW

LABORATORY FINDINGS

Test	Result	Reference Range	Status
WBC	14.2 x10 ⁹ /L	4.5 – 11.0 x10 ⁹ /L	HIGH
Neutrophils	82%	40 – 75%	HIGH
CRP	78 mg/L	< 5 mg/L	HIGH
Procalcitonin	1.2 ng/mL	< 0.5 ng/mL	HIGH
Arterial PO2	72 mmHg	75 – 100 mmHg	LOW
Arterial PCO2	38 mmHg	35 – 45 mmHg	Normal
Sputum Culture	Pending	No growth expected	—

Haemoglobin	13.8 g/dL	13.5 – 17.5 g/dL	Normal
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IMAGING & INVESTIGATIONS

Chest X-Ray	Right lower lobe consolidation; patchy infiltrates bilaterally.
CT Chest (HRCT)	Bilateral ground-glass opacities; air bronchograms in RLL; no pleural effusion.
Spirometry (pre-Rx)	FEV1/FVC 0.64; FEV1 58% predicted — obstructive pattern consistent with asthma exacerbation.

DIAGNOSIS

1	Community-Acquired Pneumonia (CAP)	RLL consolidation; elevated inflammatory markers
2	Acute Severe Asthma Exacerbation	Peak flow 55%; SpO2 93%; obstructive spirometry
3	Hypoxaemia	PaO2 72 mmHg on room air

MEDICATIONS & RECOMMENDATIONS

Amoxicillin-Clavulanate 875/125 mg	Twice daily x 7 days — empirical CAP coverage
Salbutamol (Albuterol) MDI 200 mcg	Every 4 hours via spacer — bronchodilator
Ipratropium Bromide MDI 40 mcg	Every 6 hours — anticholinergic bronchodilator
Prednisolone 40 mg	Once daily x 5 days — systemic corticosteroid
Supplemental O2	2 L/min via nasal cannula — target SpO2 > 94%

- Admitted for inpatient monitoring; review in 24 h — consider ICU if SpO2 < 90%.
- Sputum culture result to guide antibiotic de-escalation or escalation.
- Smoking cessation counselling and asthma action plan on discharge.
- Influenza and pneumococcal vaccination on recovery.

FOLLOW-UP

Follow-up Date	1 week post-discharge	Tests Required	Repeat CXR · Spirometry · Sputum result · SpO2
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Digitally prepared by Dr. Priya Nair, MD · Pulmonology
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